

Question:

A 36-year-old primigravid woman at 34 weeks' gestation comes to the physician because of a 1-week history of upper abdominal discomfort, nausea, and malaise. She had a mild upper respiratory tract infection a week ago. She has a 10-year history of polycystic ovarian syndrome and a 3-year history of hypertension. Her medications include metformin, labetalol, folic acid, and a multivitamin. Her pulse is 92/min, respirations are 18/min, and blood pressure is 147/84 mm Hg. Examination shows a nontender uterus consistent in size with a 34-week gestation. There is mild tenderness of the right upper quadrant of the abdomen. The fetal heart rate is reactive with no decelerations. Which of the following is the most appropriate next step in management? Answer Choices: A. Serum bile acid levels B. Stool antigen assay for H. pylori C. Reassurance and follow-up D. Serum transaminase levels and platelet count

Reasoning + Answer:

<think> Since the patient has pruritis and jaundice with a cholestatic liver function test and a positive family history of obstetric cholestasis the most likely diagnosis is intrahepatic cholestasis of pregnancy and the next test to do is to check the serum bile acid levels to confirm the diagnosis and monitor the severity of the condition during the pregnancy </think> <answer> A . Serum bile acid levels </answer>

Reward score

1

0

-1

-2